INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Docetaxel alone for previously treated non-small-cell lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00203
Version 7

Docetaxel by drip on Day 1, repeated every 21 days, for up to six cycles. Treatment may stop earlier if it is not helping or side effects are unacceptable. Your team confirms the prescribed course.

Treatment stages

01 **Docetaxel monotherapy**
Every 21 days

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Docetaxel alone for previously treated non-small-cell lung cancer · National Cancer Control Programme (NCCP) 00203

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle. Severe tummy pain, rapidly worsening diarrhoea, blistering or peeling skin, or new breathing symptoms also need urgent assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

Treatment aims to slow or shrink lung cancer and relieve symptoms after previous chemotherapy. Expected benefit, side effects, other options and your priorities guide the decision.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell’s division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your lung treatment plan

Exact cancer setting

NCCP 00203a: locally advanced or metastatic non-small-cell lung cancer after failure of prior chemotherapy. This is docetaxel alone; a platinum or other partner needs its own regimen.

Course and visits

Docetaxel 75 mg/m² is infused on Day 1 every 21 days. For this non-small-cell lung cancer (NSCLC) indication, the NCCP plan has a maximum of six cycles; treatment stops earlier for progression or unacceptable side effects. Your team confirms your prescribed dose and course.

What the evidence can tell us

A historical randomised trial found that docetaxel 75 mg/m² every three weeks prolonged survival compared with supportive care alone in selected people whose NSCLC had progressed after platinum chemotherapy. Modern previous treatment and available alternatives differ, so the trial cannot predict an individual outcome.

Why this treatment may be chosen

Discuss possible benefit, alternatives and uncertainty for your own situation. The treatment aim is cancer control and symptom relief.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Docetaxel alone for previously treated non-small-cell lung cancer · NCCP 00203

Treatment pattern

One 21-day cycle

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 21 DAYS

Docetaxel monotherapy

Cycle 1 | Cycle 2 | Cycle 3 | Cycle 4 | Cycle 5 | Cycle 6

Docetaxel 75 mg/m² is infused on Day 1 every 21 days. For this NSCLC indication, the NCCP plan has a maximum of six cycles; treatment stops earlier for progression or unacceptable side effects. Your team confirms your prescribed dose and course.

The team confirms your actual prescription and appointments. This map explains the pattern and does not decide whether a dose can be given.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative stages explain the order of care; they are not equally spaced appointments. full blood count (FBC), kidney and liver profile at baseline and before every 21-day cycle; review symptoms, neuropathy, fluid retention and the steroid plan. Confirm remaining cycles against the six-cycle NSCLC maximum.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00203 v7 · official regimen · EMA Taxotere product information; retrieved 2 November 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel alone for previously treated non-small-cell lung cancer: treatment and checks

Lung · previously treated advanced NSCLC

Docetaxel by drip on Day 1, repeated every 21 days, for up to six cycles. Treatment may stop earlier if it is not helping or side effects are unacceptable. Your team confirms the prescribed course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Treatment plan NCCP 00203	Confirm indication and prescription	Day 1 every 21 days	Maximum six cycles for NSCLC	Clinician review before restart	Agree the next plan
Blood / kidney / liver NCCP 00203 · SMPC	Baseline FBC, kidney and liver tests	Before each 21-day cycle	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication NCCP 00203 · SMPC	Review symptoms, medicines and reaction history	Check steroid doses, swelling and neuropathy	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing NCCP 00203 · SMPC	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course NCCP 00203 · SMPC	Confirm 00203a NSCLC after prior chemotherapy	Agree response review and actual appointment plan	Up to six cycles; stop earlier if needed	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

What each check is for

1

Safety review

Before each 21-day cycle

FBC, kidney and liver profile at baseline and before every 21-day cycle; review symptoms, neuropathy, fluid retention and the steroid plan. Confirm remaining cycles against the six-cycle NSCLC maximum.

Ask / act: The team reviews results, current symptoms and the prescription.

2

Benefit and treatment choice

At consultant-agreed response reviews

A historical randomised trial found that docetaxel 75 mg/m² every three weeks prolonged survival compared with supportive care alone in selected people whose NSCLC had progressed after platinum chemotherapy. Modern previous treatment and available alternatives differ, so the trial cannot predict an individual outcome.

Ask / act: Discuss continuing, changing or stopping treatment according to benefit, toxicity and priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the visit

Check the oral steroid schedule, blood-test appointment and transport. Contact the unit before attending if premedication was missed or you are unwell.

STEP 2

At the unit

Docetaxel is given through a cannula or suitable existing intravenous (IV) line over about one hour. The appointment also includes checks, preparation, premedication and observation and is longer than the infusion itself. This regimen has no routine take-home pump.

STEP 3

At home

Keep the contact number, medicine list and symptom record available. Ask who will arrange your next blood test, scan and review. Sedating medicines may affect driving.

Checks through treatment

Your team arranges tests, reviews results and confirms each dose.

Blood tests and symptoms

Heart and breathing

Assess cardiac history and symptoms; this monotherapy source does not prescribe routine serial echocardiograms. New breathlessness, chest pain or fainting needs assessment.

Response review

The consultant agrees when scans and other disease checks are due. New or worsening symptoms may need assessment sooner. There is no automatic fixed scan interval in this regimen.

Symptoms and supportive care

Docetaxel alone for previously treated non-small-cell lung cancer · NCCP 00203

One 21-day cycle

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Chemotherapy · Day 1	 **Day 1** intravenous (IV)
Administration reaction	 **Days 1–21** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–21** Variable timing and severity
Nerves / skin / nails	 **Days 1–21** Can build across doses

Six-cycle limit: The NSCLC source plan has at most six cycles. Progression or unacceptable side effects may stop treatment sooner.

Between visits: Symptoms may occur on any day. Do not wait for the next infusion to report urgent problems.

No dates to enter: This diagram explains the sequence; your team books actual visits and tests.

Why these bands stay broad: These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00203 v7 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1: Treatment day

Your blood results, symptoms and steroid preparation are checked before docetaxel. The infusion takes about one hour; the full visit is longer. Tell the nurse immediately about discomfort at the drip, flushing, breathing symptoms or feeling faint.

Day 2: Day after treatment

Finish the prescribed three-day steroid course and use anti-sickness medicine as directed. Ask the team if a steroid dose was missed; do not invent a replacement dose.

Day 3–20: Between visits

Watch for infection, mouth soreness, fatigue, bowel changes, numbness or swelling. Symptoms and recovery vary; no day is guaranteed to be risk-free. Contact the team promptly about worrying symptoms, even if your temperature is normal.

Day 21: Prepare for the next cycle

The team arranges blood tests and reviews recovery and benefit before another dose. Your next treatment day depends on this review. Check the next appointment and the steroid start date. Do not use a passport stamp as permission for another dose.

Support around the medicines

1 Premedication

Take dexamethasone exactly as prescribed before and after docetaxel. Tell the unit about missed doses before infusion; any alternative needs the prescriber's decision. Do not double a dose or add another steroid course yourself.

2 Nausea, bowel symptoms and pain

Use your individual anti-sickness and pain plan. Contact the team if symptoms are uncontrolled or you cannot drink. Antihistamines and other support medicines may cause drowsiness; arrange transport if advised.

3 Nerves and daily activities

Report tingling, numbness or weakness early, including effects on buttons, writing, walking or balance. Protect hands and feet and take care with hot surfaces if sensation changes.

4 Infection and blood counts

Each treatment needs its own blood and symptom review. A growth-factor injection may be prescribed after individual risk assessment; follow its separate instructions. Fever or sudden illness needs immediate contact on any day.

Practical support

Bring your medicine list and symptom record to every review.

Medicines and vaccines

Check new prescriptions, over-the-counter medicines, supplements and vaccine timing with the oncology pharmacist. Interactions and changes in organ function can alter the treatment plan.

Fertility and contraception

Discuss fertility preservation before treatment where relevant. Ask for written contraception and breastfeeding advice for your actual product and the period after treatment. Tell the team promptly about a possible pregnancy.

Support and priorities

Ask about symptom control, nutrition, rehabilitation, emotional support and help at home. Review how the treatment burden and possible benefit fit your priorities.

Questions worth asking

What benefit is realistic for me and what other options are available?

Treatment aims to slow or shrink lung cancer and relieve symptoms after previous chemotherapy. Expected benefit, side effects, other options and your priorities guide the decision. A historical randomised trial found that docetaxel 75 mg/m² every three weeks prolonged survival compared with supportive care alone in selected people whose NSCLC had progressed after platinum chemotherapy. Modern previous treatment and available alternatives differ, so the trial cannot predict an individual outcome.

Who arranges and reviews my blood tests and scans?

FBC, kidney and liver profile at baseline and before every 21-day cycle; review symptoms, neuropathy, fluid retention and the steroid plan. Confirm remaining cycles against the six-cycle NSCLC maximum. Ask the team who books each test and who checks the result.

What premedication and anti-sickness plan should I follow?

Follow the written plan for your exact prescription. Report missed premedication before infusion; confirm prevention and rescue medicines with the pharmacist.

Which symptoms need an immediate call, and which number should I use?

Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle. Keep the unit's daytime and out-of-hours numbers on your treatment card.

Try explaining the plan back: Can you explain the treatment aim, your next visit, the blood-test plan, premedication and when to call for urgent help?

Treatment scope · **NCCP 00203** NCCP 00203a: locally advanced or metastatic non-small-cell lung cancer after failure of prior chemotherapy. This is docetaxel alone; a platinum or other partner needs its own regimen.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Docetaxel · lung supportive care

Active medicines: Docetaxel

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally twice daily, unless contraindicated	For three days, starting the day before each docetaxel infusion NCCP requires a minimum of three pre-treatment doses. If doses were missed, contact the team; intravenous (IV) replacement is a consultant decision. Reconcile with any other steroid prescription.
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Dexamethasone 8 mg orally twice daily for three days starting the day before docetaxel, unless contraindicated; NCCP requires at least three pre-treatment doses. Missed doses require consultant review; 20 mg IV immediately before infusion is a discretionary alternative.
- Avoid duplicate corticosteroid prescriptions. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) may be used after individual risk assessment. Monitor fluid retention and neuropathy.
- Low emetic risk: prevention and rescue medicines remain separate from reaction premedication. Confirm individual suitability, interactions and duration; this reference is not a prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hands / feet

Docetaxel

Report numbness, weakness or difficulty walking or using your hands.

3 Hair / scalp

Docetaxel

Hair loss can occur; ask about support before treatment.

5 Mouth

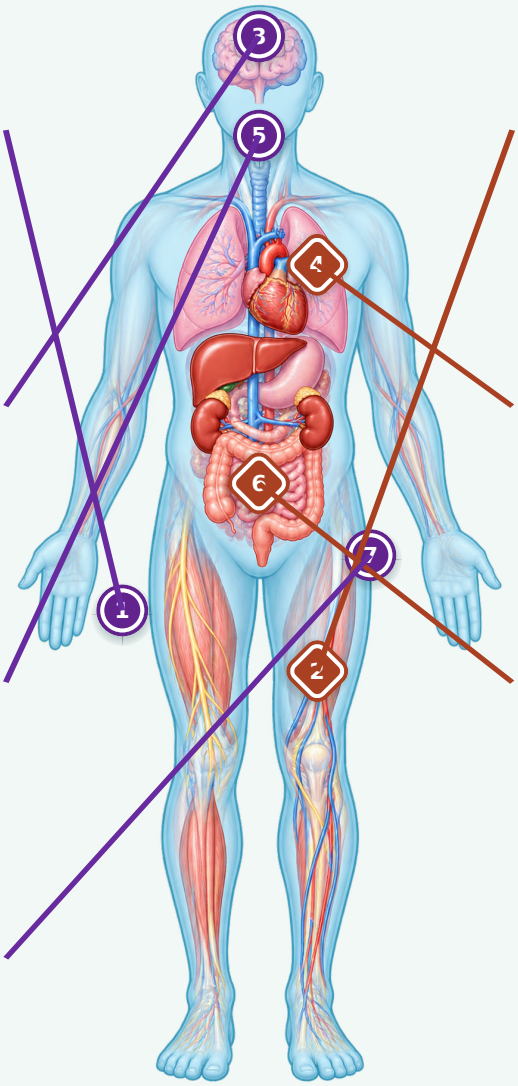
Docetaxel

Report painful ulcers, difficulty drinking or poor intake.

7 Skin / swelling

Docetaxel

Report swelling; blistering or peeling skin needs urgent advice.



2 Blood / infection

Docetaxel

Fever, shivers or sudden illness need immediate contact.

4 Breathing

Docetaxel

New or worsening cough or breathlessness needs urgent assessment.

6 Tummy / bowel

Docetaxel

Severe tummy pain or rapidly worsening diarrhoea needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle. Severe tummy pain, rapidly worsening diarrhoea, blistering or peeling skin, or new breathing symptoms also need urgent assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Docetaxel alone for previously treated non-small-cell lung cancer · National Cancer Control Programme (NCCP) 00203

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00203 v7 · official regimen**
   https://healthservice.hse.ie/documents/6543/203_DOCEtaxel_75-21day.pdf
2. **EMA Taxotere product information; retrieved 2 November 2026**
   https://www.ema.europa.eu/en/documents/product-information/taxotere-epar-product-information_en.pdf
3. **Shepherd et al., TAX317, JCO 2000; previously treated non-small-cell lung cancer (NSCLC)**
   https://pubmed.ncbi.nlm.nih.gov/10811675/
4. **NCCP NCIS medical-oncology antiemetics V8, low risk**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Fossella et al., TAX320, JCO 2000**
   https://pubmed.ncbi.nlm.nih.gov/10856094/